

Individual Case Safety Report (ICSR)

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Course: B.Pharm

1. Case Study

Case ID: ICSR-2026-001

Patient: 58-year-old male; weight 72 kg

Medical history: Hypertension and osteoarthritis; no known drug allergy reported.

Concomitant medication: Amlodipine 5 mg once daily.

Suspected medicine: Diclofenac 50 mg orally three times daily, started for acute knee pain.

Therapy start: Day 1. **ADR onset:** Day 5.

Reaction: Black, tarry stools with dizziness and weakness. Hemoglobin was found to be 8.4 g/dL. Upper gastrointestinal bleeding was diagnosed.

Action taken: Diclofenac was stopped. The patient was hospitalized, received IV fluids and acid-suppressive therapy, and was monitored for gastrointestinal bleeding.

Outcome: Patient stabilized and was discharged after 4 days. Symptoms improved after withdrawal of diclofenac. No rechallenge was performed.

2. ADR Identification

Field	Assessment
Suspected ADR	Upper gastrointestinal bleeding (melena) with symptomatic anemia and dizziness
Suspected drug	Diclofenac 50 mg oral, three times daily
Indication	Acute knee pain / osteoarthritis-related pain
Time to onset	Approximately 5 days after starting diclofenac
Dechallenge	Positive: symptoms improved after diclofenac withdrawal and treatment
Rechallenge	Not performed
Relevant risk factors	Age 58 years; NSAID exposure; possible gastrointestinal susceptibility
Outcome	Recovered / clinically stabilized after hospitalization

3. ICSR Minimum Criteria

The case contains the four core elements needed for a valid individual case safety report: an identifiable patient, an identifiable reporter, a suspected medicinal product, and a suspected adverse reaction. In a real submission, reporter identity/contact and country-specific administrative fields would be captured according to the reporting system.

ICSR Element	Case Information
Patient	58-year-old male, 72 kg
Reporter	Healthcare professional (simulated case)
Suspected product	Diclofenac 50 mg oral, three times daily
Adverse event	Upper gastrointestinal bleeding with anemia, melena and dizziness
Seriousness	Serious – required hospitalization and involved a medically important bleeding event
Outcome	Recovered / stabilized

4. Causality Assessment

Using a structured clinical assessment, the relationship between diclofenac and gastrointestinal bleeding is assessed as **Probable / Likely**. The reaction is biologically plausible for an NSAID, occurred after exposure, improved following withdrawal, and no

stronger alternative cause is provided in the case. The absence of rechallenge prevents stronger confirmation.

Causality Factor	Assessment
Temporal relationship	Compatible; reaction began after initiation of diclofenac.
Known pharmacology	NSAIDs, including diclofenac, are associated with gastrointestinal mucosal injury and bleeding.
Dechallenge	Positive; clinical improvement followed withdrawal and treatment.
Rechallenge	Not available.
Alternative causes	No major alternative cause is identified in the supplied case scenario.
Overall assessment	Probable / Likely relationship.

5. Seriousness & Severity Classification

Parameter	Classification	Rationale
Seriousness	Serious	Hospitalization was required; gastrointestinal bleeding is a medically important event.
Severity	Severe	Clinically significant bleeding with anemia, dizziness and need for inpatient treatment.
Expectedness	Expected / Listed*	The reaction is consistent with the known safety profile of NSAIDs; product-specific reference information should be checked for the actual report.
Outcome	Recovered / Stabilized	Patient improved after withdrawal and treatment and was discharged.

*Expectedness must be determined against the current applicable product information/reference safety information for the exact medicinal product and reporting jurisdiction.

6. Drug Safety Associate Assessment

As a simulated Drug Safety Associate, the case would be reviewed for validity, seriousness, medical coding, causality, expectedness and completeness. The adverse event may be coded using the applicable MedDRA terminology, with the most specific supported term selected from the reported clinical information. The case should be medically reviewed and followed up for missing information such as laboratory trends, endoscopy findings, exact dates, dose duration, prior ulcer/bleeding history, alcohol use and other medicines that may affect bleeding risk.

7. Proposed ICSR Narrative

A 58-year-old male (72 kg) with a history of hypertension and osteoarthritis was started on diclofenac 50 mg orally three times daily for acute knee pain. On Day 5 of treatment, he developed black, tarry stools accompanied by dizziness and weakness. Hemoglobin was 8.4 g/dL and upper gastrointestinal bleeding was diagnosed. Diclofenac was discontinued, and the patient was hospitalized for IV fluid therapy, acid-suppressive treatment and monitoring. His condition improved and he was discharged after 4 days. No rechallenge was performed. Based on the temporal association, known pharmacological profile and positive dechallenge, the causal relationship between diclofenac and the event is assessed as probable/likely. The case is serious because hospitalization was required and is classified as severe in intensity.

8. Reporting & Follow-up Plan

Step	Action
Case intake	Create a unique ICSR case number and verify the four minimum validity criteria.
Data entry	Capture patient, reporter, suspect/concomitant products, event, dates, treatment, laboratory results and outcome.
Medical coding	Code the adverse event and relevant medical history using the current MedDRA version.
Assessment	Perform seriousness, severity, expectedness and causality assessments.
Quality review	Check chronology, duplicate cases, missing fields and internal consistency.
Follow-up	Request relevant missing clinical information and update the ICSR when new information is received.
Submission	Transmit through the applicable national/regional pharmacovigilance reporting pathway within required timelines for serious cases.

9. Conclusion

This simulated ICSR demonstrates the workflow of a Drug Safety Associate from case intake through ADR identification, causality assessment, seriousness/severity classification, narrative preparation and follow-up. The case represents a serious and severe suspected ADR with a probable/likely causal relationship to diclofenac. Accurate, timely and well-documented case processing supports pharmacovigilance signal detection and patient safety.

References

1. ICH E2A – Clinical Safety Data Management: Definitions and Standards for Expedited Reporting.
2. ICH E2D(R1) – Post-Approval Safety Data: Definitions and Standards for Management and Reporting of Individual Case Safety Reports.
3. World Health Organization (WHO) – Pharmacovigilance guidance and training resources.
4. Uppsala Monitoring Centre (UMC) – Guidance and resources for Individual Case Safety Reports and pharmacovigilance.
5. MedDRA – Medical Dictionary for Regulatory Activities, current applicable terminology and coding principles.